

Leadership Impact Plan Paper

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Empowering Communities: A Leadership Impact Reflection on Health and Wellness

Education

The changing environment of public health and education highlights the importance of leadership that is visionary, equity-focused, and culturally aware. This Leadership Impact Plan Paper evaluates my growth, both professionally and personally, during the Health and Wellness Education program at the American College of Education. By examining essential leadership qualities, ethical considerations, and practical applications, this reflection delineates how my academic experience has prepared me with the skills needed to support others, promote resilience, and advocate for improved health outcomes in various communities.

Reflection

The key leadership qualities of my professional philosophy include empathy, active listening, and strategic vision. Empathy enables an understanding of the unique challenges faced by vulnerable populations in terms of health equity. Active listening promotes an inclusive environment where stakeholders feel acknowledged, while strategic vision allows for the conversion of research-driven data into practical wellness initiatives (Corbie et al., 2022). My graduate experience at American College of Education has significantly developed these qualities. Prior to entering this program, I viewed communication primarily as a tool for information delivery. However, advanced coursework in behavioral theory and community health intervention design fundamentally shifted this perspective. I learned that health literacy is deeply intertwined with socioeconomic, cultural, and emotional variables. To lead effectively, one must first listen deeply.

Through collaborative online discussions and peer-review processes, I was prompted to expand my viewpoint, transitioning from a prescriptive approach to a more collaborative and empowering style of leadership. Engaging with a curriculum focused on evidence-based health communication has refined my ability to convert complex health information into accessible and culturally relevant education for diverse populations (M.Ed. In Health and Wellness Education, 2026). Ultimately, these graduate experiences have transformed me into a more intentional, data-driven leader equipped to advocate for sustainable community health.

Growth

Throughout this program, my development as a leader has shifted from being an individual contributor to becoming a catalyst for systemic change. In the early stages of my journey, I often depended on my own expertise to address challenges. Currently, I utilize a shared leadership model, recognizing that lasting change necessitates the active engagement and support of my team and community members. Furthermore, over the course of this program, I have developed a deeper capacity to assess diverse needs, utilize evidence-based research, and build comprehensive health initiatives that yield tangible community impacts. Moreover, my understanding of system-level change and the social determinants of health (SDOH) has also expanded, allowing me to view health and wellness through a broader, more equitable lens.

Two notable leaders who have significantly impacted my transformation are Mikelle Moore, former Senior Vice President of Community Health at Intermountain Healthcare, and Heather Wall, Chief Commercial Officer at Civica Rx. Moore leads initiatives focused on community health, addressing critical issues such as the opioid crisis and suicide prevention. She also spearheads community-based collaborations aimed at tackling social determinants of health. By proactively addressing factors that influence individuals' health before they seek clinical

care—such as chronic hunger, affordable housing, unemployment, inadequate transportation, and safety concerns, she has shown me the power of compassionate, systemic leadership. Similarly, Wall transitioned from her role as an operations officer at Intermountain to help guide Civica Rx, a non-profit generic pharmaceutical company. Her work addresses health equity by tackling drug shortages and lowering medication costs. Wall's career trajectory has taught me the importance of innovative leadership and how strategic partnerships can disrupt broken systems to improve public health outcomes. In addition, Wall's work has shown me that true wellness education cannot exist without addressing systemic inequities. In short, both Moore and Wall's examples have inspired me to be a vocal advocate for systemic health solutions rather than focusing solely on individual behavior (Health, 2019).

Application

The knowledge gained at ACE is directly relevant to creating, executing, and assessing health education programs. For instance, by applying health needs assessment frameworks, I can design wellness initiatives tailored to the specific demographics of my community. Personally, these competencies have deepened my own self-awareness and emotional intelligence, allowing me to manage personal wellness goals and relationships more effectively. Professionally, I plan to use these enhanced skills to lead community-based workshops focused on mental health, nutrition, and chronic disease prevention. Additionally, my improved communication and team-building abilities will allow me to facilitate strong collaborations between healthcare providers, educators, and community members. By enhancing health literacy and fostering community agencies, I aim to reduce systemic health disparities and positively impact the lives of those I serve.

Ethical Perspectives

The nature of ethics in health and wellness education involves principles such as autonomy, confidentiality, and the obligation to provide fair, evidence-based care. Health educators must ensure that the information they share is scientifically accurate, culturally sensitive, and respectful of an individual's right to make informed choices regarding their well-being (Chase, 2021). As a leader, I encountered an ethical dilemma when asked to implement a mandatory workplace wellness program. The initial design unintentionally disadvantaged employees with chronic health conditions by linking health incentives to specific biometric results. Acknowledging the ethical concerns of establishing an exclusionary and potentially discriminatory environment, I advocated for a revised approach. I directed my team to restructure the program to emphasize holistic, participation-based wellness goals, guaranteeing that all employees had fair access to health resources, regardless of their starting health status.

Looking Ahead

Leadership development is a lifelong process that requires continuous, intentional effort. To further enhance my leadership skills, I intend to pursue continuous professional development through obtaining a Certified Health Education Specialist (CHES) credential, which aligns with the standards established by the American College of Education's program. I will also engage in regular mentoring, read peer-reviewed public health literature, and participate in leadership seminars focused on equity (Corbie et al., 2022). Regarding my professional goals, I aim to advance into a leadership or administrative role within a community health organization or educational institution. In this capacity, my objective is to design and manage comprehensive health education programs that emphasize trauma-informed practices and preventive care for marginalized populations.

Conclusion

My experience in the health and wellness education program at the American College of Education has deeply shaped my personal and professional development. By enhancing my knowledge of empathy, systemic equity, and ethical practice, I have become a more informed and effective leader. The skills I have gained will enable me to support others, advocate for improvements in systemic health, and promote inclusive wellness initiatives. Overall, my education has reinforced my dedication to continual learning and my determination to be an agent of change in health and wellness education.

References

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